

Prediabetes Care Navigation Context and ACCESS Alignment:

A Six Thinking Hats Analysis

Human-Centered Design White Paper — Post-Visit Patient Navigation for Diagnosis, Resource Matching, and Cost Transparency | Example Developed and Reviewed by Ann Middleton with Claude | July 2026

Introduction: The Six Thinking Hats Method

Edward de Bono's Six Thinking Hats is a structured framework for group and individual thinking that separates modes of thought so they don't interfere with one another. Rather than debating a topic from a single, blended perspective, participants deliberately adopt one “hat” at a time, focusing on a designated lens of thinking, and review the same idea through each lens in sequence. This produces a fuller, more balanced evaluation than open-ended discussion typically allows, because emotional reactions, critical risk assessment, optimistic opportunity-spotting, and creative alternatives are each given dedicated space rather than competing for attention at once.

The Six Hats at a Glance

Hat	Focus	Prompt Used in This Paper
White Hat	<i>Facts & Information</i>	Evaluate the thesis neutrally and objectively; give facts and figures.
Red Hat	<i>Feelings & Emotion</i>	Evaluate the thesis emotionally; surface positive and negative feelings and opinions.
Green Hat	<i>Creativity & Alternatives</i>	Use creative, associative thinking to generate constructive ideas and alternatives.
Black Hat	<i>Caution & Critical Judgment</i>	Evaluate the thesis critically; focus on risks, problems, skepticism, and fears.
Yellow Hat	<i>Optimism & Benefits</i>	Evaluate the thesis optimistically; point out opportunities, pluses, and realistic hopes.
Blue Hat	<i>Process & Big Picture</i>	Step back to the meta-level; classify the idea within a higher-level system.

Facilitation Resources

Teams that want to run this exercise live (individually or with an AI co-facilitator) can use the [AI-Aided Six Thinking Hats With ChatGPT](#) template on Miro, which provides a shared board for capturing each hat's output in real time. To run the Six Hats analysis against a hypothesis in Claude, visit: <https://github.com/BrionesPoppy100/six-hats-analysis>

The Thesis Under Evaluation

This white paper applies the Six Hats method to a single working thesis drawn from prediabetes clinic and patient-education workflows. Each section below presents the findings generated while wearing one hat at a time.

After a primary care visit with labs, patients need to: (1) know whether they are prediabetic; (2) understand resources available to address nutrition, physical activity, and stress, matched to their readiness for engagement and desire for social support (in community or online); and (3) know whether their employer, insurer, or health system will pay for all or part of that expense.

White Hat — Facts & Information

Objective facts and figures, evaluated neutrally.

The white hat asked what is factually known and measurable about each part of the thesis, drawing on CDC, CMS/Medicare, and peer-reviewed program-evaluation data.

- **Awareness gap is large and real:** an estimated 115.2 million U.S. adults have prediabetes, and roughly 8 in 10 are unaware of it — confirming that the thesis targets a genuine, well-documented information gap.
- **Diagnosis communication is not automatic:** independent research on primary care result-reporting found failure-to-inform rates averaging around 7%, and ranging as high as 20–26% in some practices, meaning “knowing you are prediabetic” after a visit cannot be assumed by default.
- **Readiness-matching is linked to program success:** diabetes-prevention literature identifies incorporating change-readiness into referrals, and offering pre-sessions to match participants, as evidence-informed strategies for improving retention.
- **Delivery-mode preference varies sharply by demographic group:** in a 455,954-person National DPP cohort, in-person programs enrolled more older adults and Black/African American participants, while online programs enrolled more men, younger adults, and White participants — supporting the need for both community and online options.
- **Attrition remains steep even under good conditions:** weekly attrition in CDC-recognized programs ran 1–5%, and one enrollment study anticipated roughly 50% of referred patients expressing interest, with about 20% further attrition after initial assessment.
- **Coverage exists but is inconsistent and hard to access:** Medicare covers its Diabetes Prevention Program once per lifetime at no cost, yet only about 9,000 beneficiaries used it over a six-year window, and supplier shortages mean even full site coverage in California would reach only about 10% of eligible beneficiaries annually.

Red Hat — Feelings & Emotion

Gut-level positive and negative feelings, unfiltered.

The red hat set aside justification and asked what patients might simply feel at each step of this journey.

- **Relief or validation:** for patients who sensed something was wrong, finally getting a clear answer can feel like the system took them seriously.
- **Fear, shame, or denial:** prediabetes carries a cultural association with willpower and diet, and many patients may feel self-blame or simply tune the news out.
- **Overwhelm:** a full menu of nutrition, activity, stress, community, and online options delivered at once can feel like drowning rather than empowerment.
- **Hope through connection:** patients who want social support may feel real excitement at the idea of not doing this alone.
- **Isolation or embarrassment:** patients who dread group settings may feel unseen or avoid engaging if community framing dominates without an easy solo option.
- **Financial anxiety and potential resentment:** not knowing the cost adds a distinct layer of worry, and discovering after the fact that a program wasn't covered can curdle into distrust of the health system.

Green Hat — Creativity & Alternatives

Associative, generative ideas — constructive, not yet evaluated.

The green hat generated alternative ways the thesis could be implemented, without judging feasibility.

- **A “results translator” card at checkout:** a plain-language card handed to the patient at the visit itself, rather than a delayed phone call or portal message.
- **A “choose your character” intake quiz:** a few quick either/or questions that route patients to one recommended path instead of a full resource directory.
- **A “price tag before you commit” widget:** coverage and cost shown next to each resource option, like a shopping cart, instead of a separate benefits phone call.
- **A peer “sponsor” model:** lightweight 1:1 pairing with someone further along in their own journey, borrowed from recovery-community support structures.
- **Employer-led proactive disclosure:** a one-page coverage summary sent during open enrollment, before any diagnosis occurs, so the answer is already on hand when needed.
- **Slow-drip resource delivery:** spaced, single-topic messages over time instead of one overwhelming packet, letting patients opt into depth at their own pace.

Black Hat — Caution & Critical Judgment

Risks, fragilities, and reasons for skepticism.

The black hat stress-tested the thesis for what could go wrong in execution.

- **Assumes infrastructure that doesn't exist:** real-time EHR-to-payer benefit lookups are not standard, so “know whether it's covered” requires integration work far beyond current systems.
- **Readiness assessment risks mislabeling patients:** a rushed intake could misjudge a patient as “not ready” when they're simply overwhelmed, or add clinical time that already-short visits can't spare.
- **Data show follow-through will likely be low regardless:** even with all three elements delivered well, roughly half of referred patients may never meaningfully engage, risking a program that looks complete on paper but underperforms in practice.
- **Cost transparency can discourage rather than enable:** an honest “your plan doesn't cover this” answer may close the door faster than ambiguity would have.
- **Equity risk — the model may help the already-advantaged most:** patients with more time, literacy, and stable employment are best equipped to act on all three information pieces, potentially widening disparities among populations with higher prediabetes prevalence.
- **Liability and scope-creep for the practice:** committing to deliver all three elements reliably creates documentation and legal exposure if any step is skipped, which may push clinics toward defensive over-disclosure or quiet under-promising.

Yellow Hat — Optimism & Benefits

Realistic optimism — opportunities and desirable outcomes.

The yellow hat looked for grounded reasons to believe this thesis, if executed well, creates real value.

- **Prediabetes is one of the few reversible chronic-disease diagnoses:** catching it here, while attention from the visit is still fresh, is a genuinely high-leverage moment in preventive care.
- **Matched resources can convert fear into agency:** leaving the visit with a plan tailored to readiness and preference, rather than a label and a vague suggestion, can be motivating rather than alarming.
- **Solving the cost question removes a documented major barrier:** on-the-spot clarity about coverage could meaningfully raise the number of patients who take a first step.
- **Social-support framing opens a door to genuine community:** for isolated patients, a nutrition or activity group could become a real source of connection, not just a medical intervention.
- **A working version of this model is exportable:** a successful diagnosis-resources-cost handoff for prediabetes could become a template for hypertension, cholesterol, and other chronic conditions.
- **Positive experiences generate organic advocacy:** patients who feel informed and unsurprised are likely to recommend the experience to others with similar risk factors, expanding reach beyond formal marketing.

Blue Hat — Process & Big Picture

Meta-level classification and the big picture.

The blue hat stepped back from the content to locate this thesis within larger systems of care.

- **This is a “last mile” problem, not a medical one:** the clinical detection step is already solved; the real challenge is handoff and navigation across systems.
- **It spans three institutionally separate systems:** the health system, the payer/employer, and community or digital resource providers — none of which currently owns the seam between the others.
- **It is one instance of a general “diagnosis-to-action gap” pattern:** the same three-part structure recurs across hypertension, cholesterol, and cancer-screening follow-up, making prediabetes a strong test case for a reusable model.
- **Three loosely coupled subsystems are stacked together:** a communication/notification system, a recommendation/matching engine, and an eligibility/benefits-verification system, each with different technical maturity and ownership.
- **The big picture is a shift from episodic to longitudinal care:** the thesis implicitly asks the health system to take on an ongoing relationship rather than treat diagnosis as a single, self-contained event.
- **At the highest level, this is trust infrastructure:** the unifying goal across all three components is reducing ambiguity enough that patients trust the system enough to act.

Closing Note

Taken together, the six perspectives suggest the original thesis identifies a real and well-documented gap (white hat), carries genuine emotional weight for patients on both the hopeful and fearful ends (red hat), has multiple low-cost creative entry points worth piloting (green hat), faces substantive infrastructure and equity risks that should be designed around rather than ignored (black hat), targets a moment of unusually high preventive leverage (yellow hat), and ultimately represents a broader systems-design and trust-building challenge that extends well beyond prediabetes alone (blue hat).

How the CMMI ACCESS Model Relates to These Findings

The Centers for Medicare & Medicaid Innovation (CMMI) announced the Advancing Chronic Care with Effective, Scalable Solutions (ACCESS) Model in December 2025 as a voluntary, 10-year Medicare payment and service-delivery model beginning July 2026. ACCESS tests a new reimbursement approach — Outcome Aligned Payments (OAPs) — that pays participating organizations recurring amounts to manage chronic conditions using technology-enabled care, with full payment contingent on measurable clinical improvement rather than volume of services delivered. Its Early Cardio-Kidney-Metabolic (eCKM) track explicitly names prediabetes, alongside hypertension, dyslipidemia, and obesity, as a targeted condition, with required outcome measures that include HbA1c monitoring (with end-of-period targets specifically for beneficiaries with prediabetes) and weight reduction or control (BMI below 30 or 5% weight loss). Covered activities under the model include lifestyle and behavioral support (nutrition, exercise, smoking cessation), patient education and care coordination, ordering and interpreting diagnostic tests, and monitoring of FDA-authorized connected devices, delivered in person or virtually.

Because ACCESS is still in its early implementation phase (first performance period beginning in 2026, with a Request for Applications process and payment/risk methodology details still emerging), the discussion below is necessarily provisional. It considers, hat by hat, whether and how the model touches the issues raised earlier in this paper.

White Hat — Facts: Does ACCESS Operationalize the Three-Part Thesis?

ACCESS is one of the first Medicare payment mechanisms to bundle all three elements of the thesis into a single reimbursable structure. Diagnostic test ordering/interpretation and HbA1c monitoring map to “knowing whether a patient is prediabetic”; reimbursed nutrition, exercise, and behavioral support map to the resource-matching element; and the OAP itself — a defined, published payment mechanism — is a partial answer to “will this be paid for,” at least for the Medicare fee-for-service and Medicare Advantage populations the model covers. It does not, however, address employer-sponsored insurance, which remains outside CMMI's authority and continues to depend on the employer benefit-design landscape described in the original white-hat findings.

Green Hat — Creativity: Does ACCESS Enable the Alternatives Proposed?

Several green-hat ideas find a plausible home in ACCESS's design. Its emphasis on connected devices and virtual delivery could support the “slow-drip resource delivery” concept (spaced, monitored check-ins rather than a single overwhelming packet) and the “price-tag-before-you-commit” idea, since OAP rates are fixed and published up front rather than negotiated per patient. The model's planned ACCESS Tools Directory, intended to help participating organizations identify interoperability, identity-verification, and connected-device vendors, is conceptually similar to a marketplace that could surface matched resources by delivery mode and readiness. The model does not, however, specify or require any particular patient-facing intake or readiness-matching mechanism (such as the “choose your character” quiz) or the peer “sponsor” model — these remain open design choices left to individual participating organizations.

Black Hat — Caution: What Risks Does ACCESS Introduce or Leave Unresolved?

ACCESS inherits several of the risks identified in the black-hat analysis rather than resolving them. Because at least 50% of a participant's aligned beneficiaries must meet outcome targets for full payment (for the 2026–2027 performance period), the payment structure creates a potential incentive to

preferentially enroll healthier, more engaged patients and under-serve those least likely to hit targets — the same equity concern raised earlier about the three-part thesis benefiting patients already best equipped to act. The model's data-reporting requirements (standardized APIs, a designated medical director, HIPAA/FDA compliance) add exactly the kind of administrative and infrastructure burden the black hat flagged as a barrier to smaller or safety-net organizations becoming suppliers, echoing the MDPP supplier-shortage problem documented in the white-hat section. Finally, ACCESS is a new, unproven model — outcome measures, payment amounts, and risk methodology were still being finalized as of early 2026, so its actual effect on the gaps identified in this paper is not yet demonstrated.

Yellow Hat — Optimism: What Opportunity Does ACCESS Represent?

ACCESS is arguably a direct institutional response to the systemic gap this paper identifies: a federal payment model explicitly designed to reward organizations for closing the loop between diagnosis and sustained outcome improvement, rather than paying for a single visit or a single class series. Its inclusion of prediabetes in the eCKM track, specifically requiring HbA1c and weight-related outcome tracking, sends a signal that CMS views early intervention at this stage as a priority worth restructuring payment around — consistent with the yellow-hat observation that this is a high-leverage moment in preventive care. If successful, ACCESS's 10-year runway also offers time for the kind of iterative, technology-enabled navigation tools imagined in the green hat to mature and be tested at scale.

Blue Hat — Big Picture: Does ACCESS Address the Three-System Seam?

The blue-hat analysis identified the central structural problem as three institutionally separate systems — health system, payer, and community/digital resource providers — with no single owner of the seam between them. ACCESS is a direct, if partial, attempt to construct that connective layer: it requires participating organizations to coordinate formally with a patient's existing primary care team, introduces a new CMS billing code specifically for PCP co-management and care coordination, and is designed to interoperate with existing value-based models such as the Medicare Shared Savings Program and ACO REACH. In this sense, ACCESS is best classified not as a new clinical intervention but as an attempt to build the missing coordination and financing infrastructure the blue hat described as the real underlying challenge.

Red Hat — Emotion: What Does ACCESS Leave Untouched?

Notably, ACCESS is a payment and data-reporting model, not a patient-experience design standard, so it does not directly address the emotional territory mapped in the red hat — shame, overwhelm, fear of group settings, or the desire for low-pressure social connection. These remain entirely within the discretion of individual participating organizations' program design. A meaningful gap remains between having a viable payment mechanism (which ACCESS may provide) and ensuring the human experience of receiving a prediabetes diagnosis and engaging with resources feels supportive rather than overwhelming or shaming — a reminder that policy and payment reform, on their own, do not guarantee the emotional and experiential outcomes this paper's red-hat section raised.

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